

Semax

ACTH-Derived Nootropic | 5mg & 10mg x 10 Vials

Cognitive Enhancement | Neuroprotection | BDNF | SubQ / Intranasal

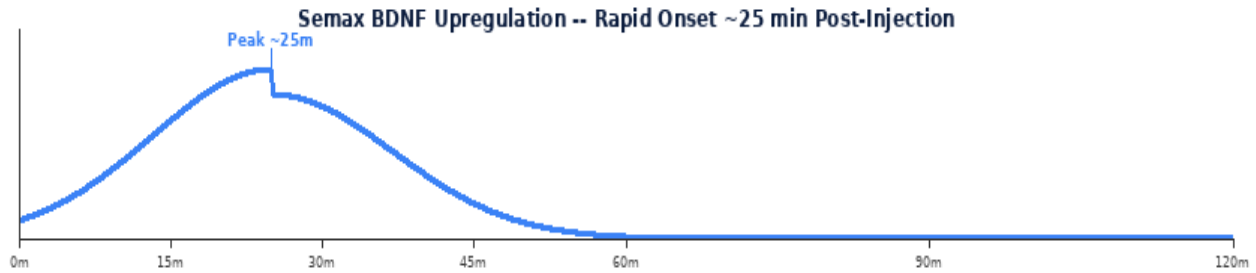
Purity: 99.8% | Endotoxin-Free | Research Use Only

Semax is a synthetic heptapeptide analogue of the N-terminal ACTH(4-10) fragment (Met-Glu-His-Phe-Pro-Gly-Pro), developed in Russia. It is a powerful nootropic and neuroprotective agent -- approved in Russia for stroke recovery, cognitive disorders, and ADHD treatment. Semax upregulates BDNF and NGF, and is available in both SubQ and intranasal forms.



Parameter	Specification
Class	Synthetic ACTH(4-10) analogue -- nootropic, neuroprotective, BDNF activator
Vial Sizes	5mg x 10 10mg x 10 Purity 99.8%
Reconstitution	1 mL BAC Water: 5mg->5mg/mL 10mg->10mg/mL
Standard Dose	200-1,000 mcg per injection SubQ intranasal also effective
Routes	SubQ Intranasal (dilute to 0.1% in saline)
Frequency	1-2x daily morning preferred no fasting required
Cycle & Storage	4-8 weeks on, 2-4 off 2-8 deg C use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
BDNF & NGF Upregulation	Rapidly upregulates BDNF (brain-derived neurotrophic factor) and NGF (nerve growth factor); promotes neuroplasticity, synaptogenesis, and neuronal survival; fastest BDNF-raising peptide in research	Cognitive enhancement; neuroprotection; neurogenesis research
Memory & Attention	ACTH(4-10) fragment modulates dopamine and serotonin systems; enhances working memory, attention, and learning speed; approved for cognitive disorders and stroke rehabilitation in Russia	Cognitive improvement; ADHD research; stroke recovery
Neuroprotection & Stroke	Approved in Russia for acute ischaemic stroke; reduces neurological deficit; neuroprotective via BDNF/NGF pathway and anti-excitotoxic mechanisms; promotes neuronal survival post-injury	Stroke; TBI; neuroprotection; excitotoxicity research

Benefit	Context
Fastest BDNF Raiser	Semax produces more rapid and potent BDNF upregulation than any oral nootropic -- effects detectable within hours.
Clinical Validation	Approved in Russia for stroke rehabilitation and cognitive disorders -- strong clinical evidence base.
Intranasal CNS Delivery	Intranasal Semax provides direct CNS delivery, reducing systemic dose needed for cognitive effects.

RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



1mL Insulin Syringe | 29-31G | SubQ

Formula: $\text{Vol(mL)} = \frac{\text{Dose(mcg)}}{\text{Conc(mcg/mL)}}$ | **Units:** $\text{Vol} \times 100$

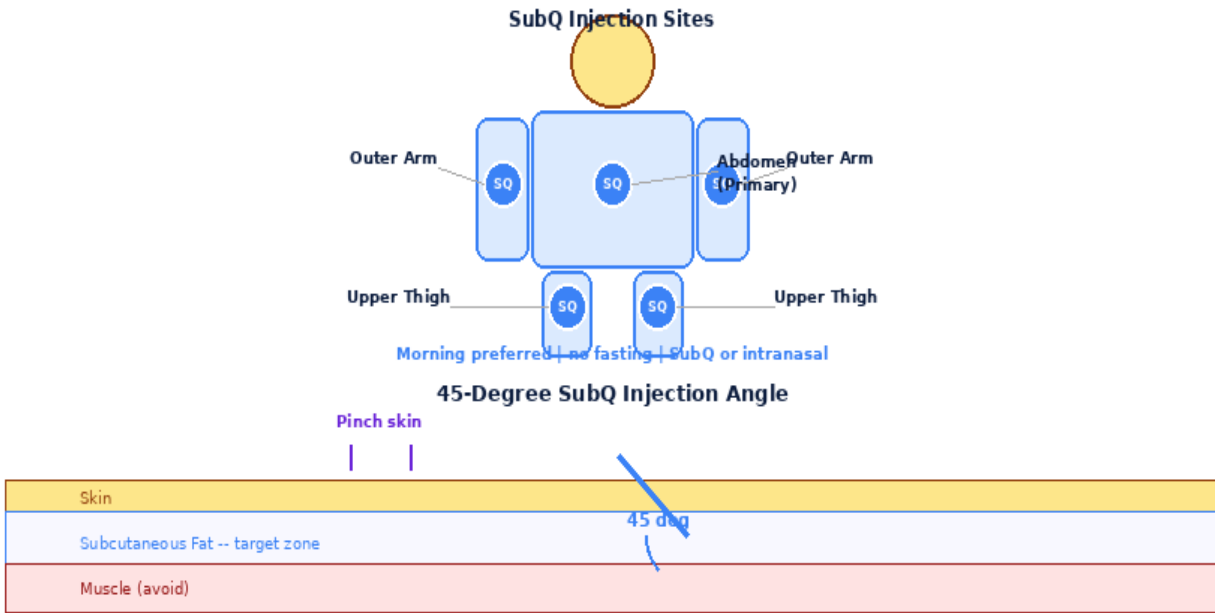
5mg x 10 -- 1mL BAC -> 5.0mg/mL (5,000mcg/mL)

Dose(mcg)	200mcg	500mcg	750mcg	1000mcg
Vol(mL)	0.0400	0.1000	0.1500	0.2000
Units	4.0U	10.0U	15.0U	20.0U
Doses/Vial	25	10	6	5

10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	200mcg	500mcg	750mcg	1000mcg
Vol(mL)	0.0200	0.0500	0.0750	0.1000
Units	2.0U	5.0U	7.5U	10.0U
Doses/Vial	50	20	13	10

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Select route	SubQ (29-31G) or intranasal (dilute 0.1% in saline).
2	Draw/prepare	SubQ: draw dose. Intranasal: dilute in saline for nasal spray.
3	Administer (morning)	SubQ: abdomen 45 deg. Intranasal: tilt, spray, sniff. Morning preferred.
4	Rotate & dispose	Rotate SubQ sites. Sharps container.

Protocol	Dose	Frequency	Duration	Notes
Cognitive Enhancement	200-500 mcg	1-2x daily (morning)	4-8 wk 2-4 off	Morning injection for optimal cognitive effect.
Neuroprotection Research	500-1,000 mcg	1-2x daily	4-8 wk	Higher dose for neuroprotection protocols.

Side Effect	Likelihood	Management
Stimulant effect	Intended	Cognitive activation. Avoid late evening injection.
Headache	Occasional	Dose-related. Reduce dose.
Irritability	Rare	Dopaminergic activation. Reduce dose.

Contraindications: Cancer history, pregnancy, under 18.

1. 5mg: 1mL BAC->5mg/mL 10mg: 1mL->10mg/mL.
2. No fasting required. Morning injection preferred for cognitive effects.
3. SubQ or intranasal -- intranasal provides direct CNS delivery.
4. Intranasal: dilute to 0.1% (1mg/mL) in saline. Standard nasal spray device.
5. Avoid late evening injection -- cognitive activation may interfere with sleep.
6. Most potent BDNF/NGF upregulator available for research.
7. Cycle 4-8 weeks on, 2-4 weeks off.
8. Store 2-8 deg C. Use within 28 days.
9. Not for use during pregnancy, lactation, or in individuals under 18.

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